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February 23, 2026

U.S. Department of Health and Human Services
Assistant Secretary for Technology Policy (ASTP) and
Office of the National Coordinator for Health Information Technology (ONC)
Attention: Request for Information:
HHS Health Sector AI RFI
Mary E. Switzer building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

RE: Docket No. RIN 0955-AA13 -- Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Sir or Madam:

We submit these comments on behalf of Cook Group Inc. ("Cook"). Cook is a family-owned group of domestic and international corporations engaged in the manufacture of diagnostic and therapeutic products for use in various medical specialties including interventional radiology, cardiology, vascular surgery, critical care, gastroenterology, urology and surgery. We are driven by medical innovation that benefits patients and by the deep connections we build to support the communities in which we operate. Our company employs about 12,000 people around the world. Eight thousand of those employees are based in the United States and while more than 53 percent of our sales are used outside the United States, approximately 70 percent of our products are manufactured in this country.

We thank the Department of Health and Human Services (HHS) for the opportunity to provide comments on this health sector Artificial Intelligence (AI) request for information. The Department is seeking public feedback on the actions it can take to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care. Further, HHS seeks feedback on ways in which adoption of AI in clinical care can be most effectively applied to support public trust and confidence in modern technology solutions, to reduce uncertainty that impedes AI innovation, and to align federal incentives so that AI is deployed in transparent ways that enhance productivity, reduce burden, lower health care costs, and improve health outcomes for patients, caregivers, and communities.

Artificial intelligence represents a significant opportunity to advance medical technology innovation and improve patient access to safe and effective care. Across the medical device lifecycle, AI has the potential to accelerate design and development,

enhance diagnostic accuracy, enable more personalized therapeutic solutions, and support more efficient quality clinical decision-making. In clinical practice, AI-enabled technologies are already demonstrating measurable value—for example, advanced interventional MRI (iMRI) systems that incorporate AI-driven image analysis techniques may support the identification of smaller or earlier-stage lesions, establish accurate procedural precision and enable more timely interventions, driving better patient outcomes and efficiency in care delivery. When responsibly developed and deployed, AI can help translate innovation into real-world clinical benefit for patients while strengthening safety, quality, and system performance.

Regulatory Frameworks

Access to high-quality, interoperable clinical data is foundational to effective AI development and deployment. HHS should promote policies that enable secure data sharing and interoperability while maintaining strong, practical privacy protections under HIPAA. Overly prescriptive, duplicative or rigid privacy controls risk limiting the availability of data needed to train, validate, and monitor AI systems. **A balanced regulatory framework is essential to supporting responsible data use while accelerating innovation and preserving patient trust.**

Payment and Incentives

We support payment policy changes that ensure payers have the incentive and ability to promote access to high-value AI clinical interventions, foster competition among clinical care AI tool builders, and accelerate access to and affordability of AI tools for clinical care.

Current reimbursement models often fail to recognize the value created by AI-enabled clinical care, particularly where AI improves outcomes, efficiency, or clinician workload without increasing service volume. HHS should explore reimbursement pathways that reward demonstrated value, including new or updated codes, bundled payments, or value-based payment models that account for AI-driven improvements. Aligning reimbursement with innovation incentives is critical to scaling AI adoption across health care systems.

Research, Evidence and Learning Systems

Recognizing that HHS supports one of the world's largest health research ecosystems, HHS should support structured public-private partnerships to generate real-world evidence on AI performance, safety, and equity impacts. HHS should invest in federated research networks, shared testbeds, and pilot programs which would enable scalable evidence generation and development while protecting proprietary intellectual property. These public stakeholder collaborations would accelerate learning, support responsible adoption, and provide regulators and providers with credible, real-world insights into AI effectiveness.

Artificial intelligence (AI) has the potential to meaningfully improve patient care, enhance safety, and strengthen clinical and regulatory decision-making across the health care system. Realizing this potential depends not on replacing existing regulatory or clinical frameworks, but on modernizing the data and evidence infrastructure that supports them.

The recommendations outlined below are not intended to shortcut FDA review, weaken standards for medical device quality, or displace the role of clinicians, regulators, or other professionals entrusted with protecting patients. Rather, they are intended to enable existing oversight frameworks to function more effectively in an increasingly data-driven health care environment.

From our experience across the medical technology ecosystem, **there are three interrelated areas where improved interoperability and policy alignment are essential to support responsible AI adoption in health care:** 1) adoption of the medical device Unique Device Identifier (UDI) in data capture, 2) use of real-world evidence (RWE) for regulatory decision-making, 3) timely, evidence-aligned coverage and payment decisions, and 4) governance, accountability and liability clarity.

1. Adoption of UDI Data Capture by Health Care Providers and CMS

- UDI capture is foundational to trustworthy AI in health care. Improved interoperability can enable reliable device-level evidence through structured UDI capture within electronic health records, registries, and claims systems. While UDI requirements are well established at the point of device labeling, downstream capture within clinical systems and administrative data remains inconsistent.
- UDI data capture is particularly important for higher-risk devices used across diverse patient populations and care settings.

2. Use of Real-World Evidence for Regulatory Decision-Making

- Real-world evidence can complement traditional regulatory pathways when data are relevant and reliable. Use of real-world evidence does not reduce regulatory rigor; it supports decisions grounded in real-world clinical performance.
- Interoperability enables scalable, representative evidence generation across clinical records, imaging systems, registries, and administrative data sources.
- AI-enabled evidence generation can also serve existing FDA programs designed to expedite patient access to transformative technologies. For example, the FDA's Breakthrough Devices Program provides an established framework for early engagement and iterative evidence development for devices addressing unmet medical needs.
- AI-supported real-world data analysis can strengthen the Breakthrough Devices Program by enabling more efficient evidence generation, supporting postmarket learning, and accelerating the translation of technological innovations into clinical practice without compromising regulatory rigor.

- AI-enabled analytics can support lifecycle-based regulatory oversight through early signal detection and performance monitoring.
- Use of real-world evidence does not reduce regulatory rigor; it supports decisions grounded in real-world clinical performance.

3. Enable Timely, Evidence-Aligned Coverage and Payment Decisions

- Misalignment between regulatory evidence, coverage decisions, and payment policy is a known limitation of our current systems that often delay by several years patient access to beneficial technologies.
- Enhanced interoperability and reliance on real-world evidence can support more timely and informed coverage and reimbursement decisions while maintaining the highest standards of safety, quality, and effectiveness.
- **Our shared objective as a health care ecosystem should be to ensure that beneficial technologies reach patients in time—not only to improve quality of life and manage chronic disease, but in many cases to save lives—while preserving the rigor, accountability, and trust that underpin public confidence in medical innovation.**

4. Governance, Accountability, and Liability Clarity

- Ongoing uncertainty around responsibility for AI-enabled tools across developers, deploying organizations, and clinicians continues to slow responsible adoption. HHS can support innovation and patient safety by issuing practical governance guidance that clarifies accountability across the AI lifecycle, rather than creating new liability regimes.

Effective oversight of artificial intelligence must enable innovation while ensuring patient safety, public trust, and regulatory clarity across the health care ecosystem.

- **Appropriate Regulation**

AI regulation should focus on the *specific use and application* of AI rather than the underlying technology itself. Oversight should be risk-based, proportionate to potential patient impact, and capable of adapting over time as technologies and clinical use evolve.

- **United States Leadership in AI**

The U.S. should lead the world in responsible AI development by establishing a clear, coherent federal regulatory framework that supports innovation and can be harmonized internationally. Global alignment is critical to advancing patient access, fostering innovation, and avoiding conflicting requirements. **A federal approach to AI oversight in health care is essential to avoid a fragmented regulatory landscape that could impede innovation and limit patient access.**

Inconsistent or conflicting state-level requirements risk creating uncertainty, duplicative compliance obligations, and uneven adoption across the health

system. Federal standards that provide clarity, consistency, and appropriate preemption in areas of national importance will help ensure that AI-enabled medical technologies can be developed, evaluated, and deployed responsibly at scale, while preserving patient protections and public trust.

- **Health Care AI Alignment**

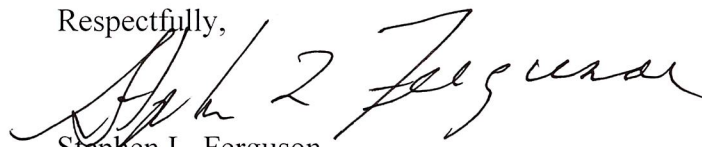
Regulation of AI in medical technology and health care applications should align with existing FDA, ISO, and other established regulatory standards to prevent unnecessary duplication and compliance burden. Oversight should remain focused on improving patient outcomes, patient safety, and access to care, while ensuring protection of personal health information consistent with HIPAA requirements.

- **Transparency and Trust**

Transparency should enhance public trust by supporting appropriate validation, monitoring, and accountability of AI-enabled systems, while protecting proprietary information and trade secrets. Transparency requirements should be designed to avoid unnecessary compliance burdens that could impede innovation without improving patient safety.

Thank you for issuing this request for information on this topic and for providing an opportunity to comment on this important public health matter.

Respectfully,

A handwritten signature in black ink, appearing to read "Stephen L. Ferguson", written over a horizontal line.

Stephen L. Ferguson
Chairman of the Board